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Bwrdd Iechyd Prifysgol Betsi Cadwaladr University Health Board

ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT DIRECTORATE

OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

ANAESTHETIC REFERRAL & ASSESSMENT OF THE MORBIDLY OBESE (BMI >40) PARTURIENT

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1. Introduction and Purpose

It is important that patients with morbid obesity are referred to the anaesthetist in advance of delivery so that they can be adequately assessed. This is with particular regard to assessing their airway. In addition, advice can be given regarding regional anaesthesia for their delivery.

2. Scope

This guideline is intended as a reference for obstetricians & anaesthetists.

3. Points to consider

- All morbidly obese patients with a booking BMI > 40 should be referred by the obstetric team for an antenatal anaesthetic assessment before 36 weeks gestation.

- For antenatal inpatients with a BMI > 40, the antenatal anaesthetic assessment should be undertaken by the duty obstetric anaesthetist (Pager 551).

- For antenatal outpatients with a BMI 40 – 49.9, the antenatal anaesthetic assessment should be undertaken by the duty obstetric anaesthetist (Pager 551) on the Maternity Outpatient assessment Unit (MOAU) when the patient attends their first consultant antenatal clinic appointment. In this case the obstetrician should arrange for the patient to attend the MOAU for anaesthetic assessment following their antenatal clinic appointment.

- If the duty obstetric anaesthetist undertaking the assessment is a trainee or speciality anaesthetist, and they identify issues of significant concern, then the case should be discussed with the Consultant Anaesthetist covering Labour Ward (Pager 170 or 642).

- For antenatal outpatients with a BMI > 50, the obstetrician should refer the patient to the antenatal anaesthetic clinic where a Consultant Obstetric Anaesthetist will undertake a face to face assessment of the patient.

- All morbidly obese patients with a BMI > 40 should be directed to the ***Labourpains.org*** website for information on anaesthesia for Caesarean Section & pain relief in labour. They should also receive a hard copy of the ***'Anaesthetics & Pregnant Women with a High BMI'*** leaflet.

- The patient will in addition be reviewed by the labour ward duty anaesthetist on arrival in labour or alternatively assessed on the day of their elective Caesarean Section by the anaesthetist undertaking the list.

- On arrival of the patient on labour ward, the duty obstetric anaesthetist (Pager 551) should be paged by the midwife to come and assess the patient. They should discuss early epidural analgesia, and in particular its benefits in morbid obesity. If the patient's airway is of concern then duty obstetric anaesthetist should contact the Consultant Anaesthetist covering Labour Ward (Pager 170 or 642) for advice.

4. References

1. Lewis,G (ed) 2007. Confidential Enquiry into Maternal and Child Health (CEMACH). Saving Mothers' Lives: reviewing maternal deaths to make motherhood safer- 2003-2005.